Documentation Gap Analysis: TGA COVID-19 Vaccine Safety Monitoring Plan Audit

INDEPENDENT REGULATORY COMPLIANCE ANALYSIS

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Executive Summary

Audit Scope & Methodology

Comprehensive evidence-based assessment of TGA's February 2021 COVID-19 Vaccine Safety Monitoring Plan implementation across 19 specific measurable outputs. Analysis based on the Department's sample searches reported to the OAIC in September 2024 (FOI Act s55V(2)), FOI responses, and public documentation spanning four years (2022-2025).

3
Fully Implemented (16%)

13
Partially Implemented (68%)

3
Not Documented (16%)

Key Finding: While monitoring activities occurred (evidenced through 150+ weekly safety reports), systematic documentation of HOW enhanced COVID-19 vaccine safety monitoring was conducted is absent. Zero coordination protocols found linking AusVaxSafety's 6.8M surveys to TGA's 150 signal investigations. This creates an accountability gap: activity ≠ systematic implementation.

Complete Documentation Breakdown by Strategy:

- Strategy 1 (Reporting of AEFI): 40% fully documented
- Strategy 2 (Signal Detection and investigation): 0% fully documented as enhanced
- Strategy 3 (Regulatory Actions): 0% fully documented
- Strategy 4 (Communication): 100% fully documented - best performance
- Strategy 5 (Collaboration): 0% fully documented
- Governance: 0% fully documented - critical gap

The Australian COVID-19 Vaccination Policy, endorsed by National Cabinet on 13 November 2020, provided for COVID-19 vaccines to enter through the provisional approval pathway, accompanied by a Commonwealth commitment to enhanced safety monitoring through the TGA's COVID-19 Vaccine Pharmacovigilance Plan (later titled the COVID-19 Vaccine Safety Monitoring Plan).

Audit Methodology Framework

Evidence-Based Approach

- Gap analysis comparing Feb 2021 Plan commitments against documented implementation.
- Assessment against 17 specific measurable plan strategies.*
- Evidence hierarchy: primary statutory (FOI, OAIC, TGA communications) > primary documentary (Senate testimony, QON 559, reports) > secondary documentary > tertiary contextual.
- Objective evaluation: What can be demonstrated vs what cannot through temporal analysis over four years.
- Falsifiable and replicable research framework.

Primary Evidence Sources

- TGA OAIC Submission MR22/00538 (Sept 2024): Sample searches organised by implementation plan objectives under legal obligation FOI Act s55V(2).
- Senate testimony (9 Oct 2025): Official statements under parliamentary privilege; QON 559 (Oct 2025).
- FOI responses (2022-2025): Formal government records spanning 4 years.
- TGA public documentation: 150+ weekly safety reports, regulatory decisions, annual reports .

Audit methodology applied systematic, evidence-based principles consistent with ISO 19011 (management systems auditing), ISO 15489 (records management), and Australian National Audit Office compliance audit standards

Implementation Status Classification Criteria

✓ Fully Implemented: Multiple forms of evidence, publicly accessible, systematic documentation exists, can be independently verified.

⚠ Partially Implemented: Some activity or process documentation exists, but outputs, results or systematic implementation against the Plan cannot be verified.

✗ Not Documented: No documentation of systematic implementation found in FOI releases or the Department's reported searches, despite claimed activities.

*Two governance outputs were assessed alongside the Plan's 17 pharmacovigilance strategies, making 19 outputs audited in total

Evidence Hierarchy

TIER 1: Primary Statutory Evidence

FOI decisions (including FOI 3643), OAIC submissions (MR22/00538, MR25/01153), and direct TGA communications to the applicant. Formal statutory positions under the FOI Act 1982 and OAIC oversight.

TIER 2: Primary Documentary Evidence

Senate Community Affairs Legislation Committee testimony under parliamentary privilege (9-10 October 2025); Senate Question on Notice 559 (SQ25-001584); TGA Annual Reports 2022-23 and 2023-24; published COVID-19 vaccine safety reports; the COVID-19 Vaccine Safety Monitoring Plan (February 2021).

TIER 3: Secondary Documentary Evidence

Partner organisation reports (NCIRS AEMS COVID-19 vaccines annual report; AusVaxSafety surveillance data); Australian Immunisation Register data; publicly accessible surveillance data and performance metrics.

TIER 4: Tertiary Contextual Evidence

International regulatory guidance (EMA, MHRA, FDA enhanced monitoring frameworks); pharmacovigilance standards (ICH E2E pharmacovigilance planning; CIOMS signal management); comparative regulatory frameworks.

Higher-tier evidence takes precedence in case of conflict. All findings are anchored in Tier 1 or Tier 2 evidence.
Findings rest on the TGA's own searches and statements, not the auditor's interpretation.

Key Assumptions & Limitations

Key Assumptions

1. Good Faith Searches: TGA's September 2024 searches under OAIC direction conducted in good faith using appropriate search terms.

2. System Coverage: TRIM and Promap document management systems contain implementation documentation if it exists.

3. Classification Framework: TGA's own plan objective classification (used Sept 2024) is appropriate framework.

4. Absence Significance: Nil results from the Department's sample searches, together with FOI releases and public records, indicate absence of documentation of systematic implementation, not mere difficulty locating.*

5. Under legal obligation, the Department reported sample searches (TRIM and Promap) and estimated more than 2,218 pages as relevant to the scope of the five Plan objectives. No search term identified documents evidencing implementation of the Plan.

6. Activity vs Documentation: Absence of documentation doesn't mean activities didn't occur, but does mean implementation of enhanced systemic post-market safety monitoring cannot be verified.

7. Temporal Validity: Evidence from 2021-2026 remains valid indicator of implementation patterns.

Limitations

1. Search Completeness: Reliant on TGA's own searches; cannot independently verify all TGA systems were searched.

2. Document Sampling: Analysis based on documents TGA identified; search categories, additional documents may exist but not located.

3. Informal Documentation: Assessment focuses on systematic documentation; informal records or institutional knowledge not captured.

4. Verification Constraints: Cannot independently audit TGA's internal document management practices.

5. Temporal Gaps: Analysis covers 2021-2025; implementation activities may have been documented after September 2024 searches.

6. Legal Interpretation: Assessment identifies patterns and contradictions; does not constitute legal determination of statutory breach (requires judicial assessment).

*No documentation identified in TGA sample searches, FOI responses, or public records

TGA's Regulatory Guidance - Provisional Approval

TGA Provisional Registration Transition Guidance

TGA's own published guidance establishes requirements for provisional approval pathway

TGA Provisional Approval Requirements

Enhanced Safety Monitoring Requirements

Provisional approval is granted where further data is needed. Enhanced safety monitoring accompanied the provisional pathway, where sponsors may be subject to conditions requiring ongoing evidence generation, pharmacovigilance and post-market monitoring.

Risk Management Plans

Sponsors must submit detailed risk management plans setting out specific monitoring activities, timelines, and deliverables, addressing the uncertainties in preliminary clinical data.

Additional Studies Required

The TGA may require sponsors to conduct additional studies during the provisional period to address data gaps. Confirmatory data must establish the relationship between preliminary outcomes and clinical benefit.

Performance Conditions

Provisional registration is time-limited on the ARTG and subject to conditions that must be satisfied. Transition to full registration requires the sponsor to demonstrate that those conditions have been met.

Regular Reporting

Sponsors must provide regular safety and efficacy updates throughout the provisional period, with closer coordination across clinical, nonclinical, quality, and RMP evaluation areas.

TGA Evaluation and Delegate Satisfaction

The TGA evaluates the data submitted by the sponsor, and the Delegate must be satisfied that provisional conditions are met before granting transition to full registration. The TGA may seek external expert advice more frequently during the provisional period.

TGA's Accountability Gap

No Enhanced COVID-19 Vaccine Safety Monitoring Evidence

TGA published its COVID-19 Vaccine Safety Monitoring Plan (Feb 2021) committing to 'enhanced' monitoring. TGA investigated 148–150 signals and took 57 actions (Senate/QON 559, Oct 2025), but senior TGA official in Senate characterised this as 'day-to-day processes'. No documentation shows activities were enhanced beyond standard pharmacovigilance, and no audit trail links specific signals to specific actions, as expected under international good practice (ICH E2E, CIOMS).

No Plan-Level Transition Verification

Comirnaty (July 2023) and Spikevax (April 2023) transitioned on the sponsor's Category 1 application and benefit-risk data. The burden sits with the sponsor to demonstrate compliance. No documentation evaluating either transition against the Feb 2021 Plan's enhanced-monitoring conditions was located.

Contradictory Characterisation

Plan (Feb 2021): 'enhanced' monitoring. Senate testimony (Oct 2025): 'day-to-day processes'. These cannot both be operationally true as stated. If day-to-day satisfied 'enhanced', the distinction promised under the Vaccination Policy has no content.

Sponsor-Demonstrated Compliance, No Plan Trail

TGA guidance gives provisional medicines 'high priority' under an enhanced framework, with the burden on the sponsor to 'continue to demonstrate' compliance or registration lapses. No documentation was located showing how the Plan's enhanced commitments were assessed at framework level during the provisional period or at transition.

Unverifiable Against the Plan's Commitments

The Feb 2021 Plan was the basis for enhanced monitoring across 68.4 million provisional doses. On the documentary record — FOI, OAIC-directed searches, published reports — implementation of that enhanced framework cannot be independently verified.

Provisional Approval Lifecycle - Verification Gap

Provisional Approval Lifecycle Framework
Legislative Requirements (TGA Act 1989)

S 22C-s 22D: Determination Phase

Sponsor applies for a provisional determination (s 22C); the TGA assesses eligibility and may grant a provisional determination (s 22D), based on preliminary data and development stage.

S 23AA & s 28(2A)(aa): Provisional Registration

An application with an in-force provisional determination is treated as an application for provisional registration (s 23AA). The TGA may impose conditions requiring ongoing evidence generation, pharmacovigilance, and post-market monitoring during the provisional period.

s 29: Extension of Provisional Registration

Provisional registration is time-limited. Up to two extensions of two years each are permitted (s 29(4)), to a six-year maximum (s 29(8A)).

s 23 & s 25: Transition to Full Registration

Transition requires the sponsor to lodge a new application (s 23). The TGA evaluates the clinical dataset (s 25) to determine whether the medicine meets the baseline standards of safety, quality and efficacy required for full ARTG registration.

TGA Guidance Requirements

Risk Management Plans (RMPs)

Sponsors must submit detailed RMP outlining safety specification, pharmacovigilance activities, risk-minimisation measures, timelines and milestones for provisional period.

Transition to Full Registration

Requires the sponsor to demonstrate that provisional conditions have been met, including completion of confirmatory studies imposed as conditions of registration and delivery of updated safety and efficacy data. The TGA evaluates the transition application against the standards for full registration.

Scale and Significance

68.4 million doses (91.2% of rollout) administered under provisional approval between February 2021 and July 2023—Australia's largest deployment of provisionally approved medicines.

Evidence Gap: No Verification Documentation Found

MR22/00538 Searches (September 2024)

The Department's sample searches (TRIM and Promap) used eight generic terms under OAIC direction. No documents were located demonstrating:

- Verification that enhanced monitoring commitments were delivered before transitions
- TGA's documented assessment of whether provisional conditions met
- Decision-making criteria or rationale for granting full approval
- Verification checklist or evaluation of implementation against February 2021 Safety Monitoring Plan

FOI 25-0166 (June 2025): TGA committed to enhanced surveillance beyond routine processes during provisional approval. Sample searches (Sept 2024) estimated 2,218+ pages but found no Plan implementation records. TGA estimated 399+ documents, then refused the narrowed request. A senior TGA official called the Plan's themes 'day-to-day processes'; no systematic tracking found. No protocols linking AusVaxSafety's 6.8M surveys to TGA's 150 signals were located. Enhanced monitoring unverified. (Strategy 2; Senate Oct 2025; QON 559; FOI 25-0166).

AusPAR Analysis: No documented verification of the TGA-led enhanced monitoring framework was identified in the reviewed transition records.

Comirnaty AusPAR (July 2023): Documents conversion to full registration (TGA decision 13 July 2023; ARTG entry 14 July 2023) but identifies no evaluation of Safety Monitoring Plan implementation or verification of the enhanced monitoring framework.

Spikevax transition (May 2023): TGA transition materials, including the approval announcement and subsequent AusPAR, do not identify verification of implementation of Safety Monitoring Plan.

Pattern: AusPARs summarise clinical data, risk management plans and registration conditions but do not identify a verification checklist or evaluation of implementation of the enhanced monitoring framework.

Critical Accountability Gap: No evaluation against the February 2021 Plan's enhanced-monitoring commitments is identified in the public or FOI record reviewed. TGA's approval process describes post-market monitoring but does not identify verification of Plan implementation before transition. While the TGA evaluated sponsors' transition applications against the general full-registration standards (including safety, quality and efficacy requirements under s 25), no documented process demonstrates how implementation of the Plan's enhanced-monitoring commitments was assessed as part of the transition process. The evidentiary basis for independently verifying delivery of those TGA-led monitoring activities across 68.4 million provisionally registered doses cannot be established from the available documentary record.

International Comparison - Regulatory Transparency Standards

TGA's disclosable record omits documentation routinely published by peer regulators

FDA Practice (US)

- Approval letters detailing basis for decision.
- Comprehensive review memoranda from FDA medical officers.
- Clinical study data summaries.
- Post-authorisation safety monitoring reports.
- Briefing documents for advisory committee meetings.
- Live streaming of Public advisory committee proceedings (VRBPAC).
- Example: Comirnaty approval (Aug 2021) = 30-page approval letter + hundreds of pages of supporting documentation.

EMA Practice (EU)

- European Public Assessment Reports (EPARs) for each transition.
- Pharmacovigilance Risk Assessment Committee (PRAC) recommendations.
- Periodic safety update reports.
- Committee for Medicinal Products for Human Use (CHMP) assessment reports.
- Comprehensive safety monitoring results.
- Independent expert committee review documentation.

TGA Practice (Australia)

Published Documentation:

- Australian Public Assessment Reports (AusPARs) – published summaries of TGA's evaluation of sponsor submissions, including clinical data, key benefit–risk considerations .
- Media releases announcing transitions and related decisions.

Notably Absent (in public and FOI record):

✗ TGA's own post-market monitoring results and analysis.

✗ Systematic evaluation of compliance with provisional-approval conditions, including in TGA annual performance reports.

✗ Safety Monitoring Plan implementation assessment.

✗ Documentation of how the Plan's enhanced monitoring commitments were verified.

No independent expert review:

No external advisory review identified: The Comirnaty transition record does not identify referral of the transition application to the Advisory Committee on Vaccines.

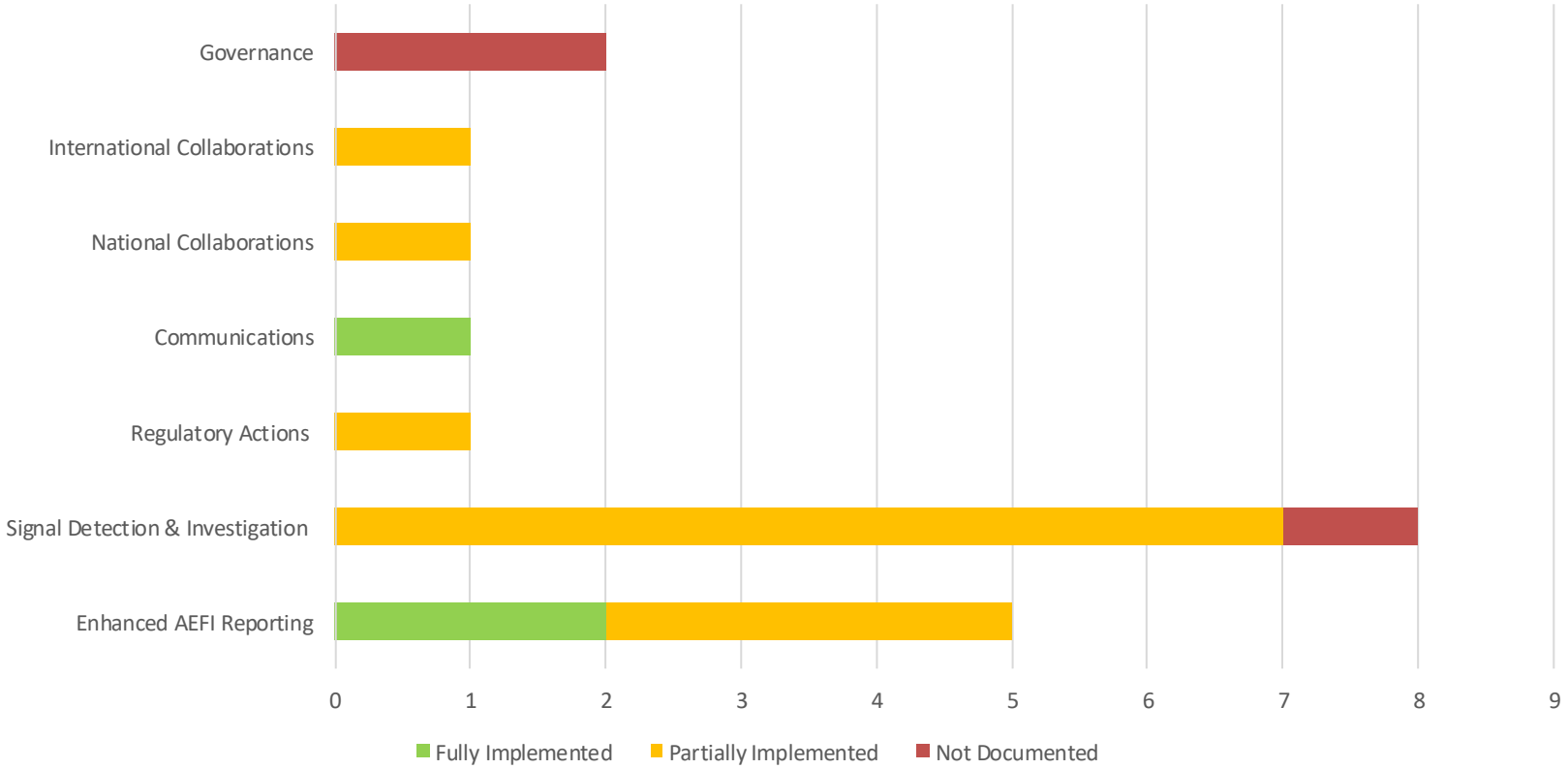
Implementation Status by Safety Plan Outputs

Strategy	Total outputs	Implemented	Partial	Not Documented
1: Enhanced reporting of Adverse Events Following Immunisation	5	2 (40%)	3 (60%)	0 (0%)
2: Enhanced safety signal detection and investigation	8	0 (0%)	7 (87.5%)	1 (12.5%)
3: Actions in response to safety concerns	1	0 (0%)	1 (100%)	0 (0%)
4: Communications	1	1 (100%)	0 (0%)	0 (0%)
5: Collaborations	2	0 (0%)	2 (100%)	0 (0%)
Governance and performance measurement*	2	0 (0%)	0 (0%)	2 (100%)
Total	19	3 (16%)	13 (68%)	3 (16%)

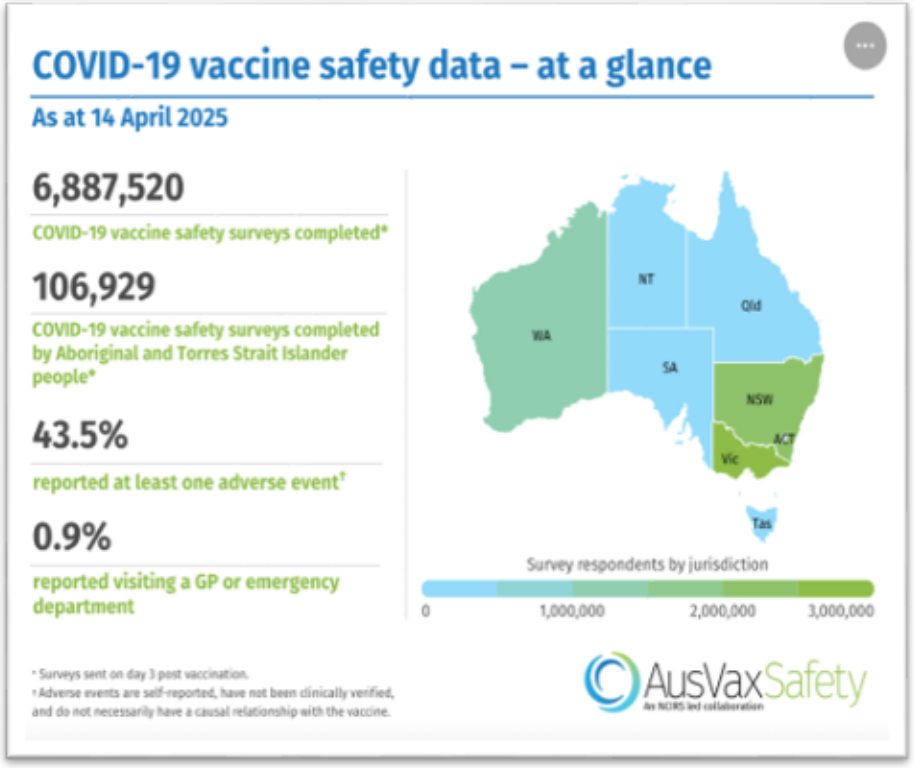
*Assessed using ISO 19011:2018 audit method. Two governance outputs (GOV.1, GOV.2) added against PGPA Act 2013 ss 37-39, additional to the Plan's 17 strategies.

Fully documented: fully evidenced in official documentation, Partial: some activity, but incomplete documentation; Not Documented: no evidence found

Implementation Status by Safety Plan Strategy



AusVaxSafety: Active Surveillance Without Documented Integration



Critical Documentation Gap:

- Zero coordination protocols found linking AusVaxSafety data to TGA's 150 signal investigations.
- No documented integration between active surveillance (6.8M surveys) and passive surveillance (DAEN database).
- No audit trails showing how population-level adverse event data informed TGA's 57 regulatory actions.
- Cannot verify disposition of 93 signals resulting in "no further action".

Why This Matters:

- Active surveillance promised as core pillar of TGA-led pharmacovigilance in Australian COVID-19 Vaccination Policy.
- Strategy 2.4 commitment: "Collaborate with AusVaxSafety to receive active surveillance information and coordinate safety signal detection and investigation activities".
- Without documented integration frameworks: Cannot verify whether 62,000 medical attention reports and 3 million adverse event reports systematically informed signal prioritisation and regulatory decisions.
- International benchmark: Pharmacovigilance standards (ICH E2E, CIOMS VIII, EMA GVP IX) emphasise documented signal management, coordination and traceability—not parallel surveillance alone.

Key finding: Operational capability ≠ enhanced COVID-19 vaccine safety monitoring framework – infrastructure functioned, but systematic integration is undocumented and remains unverified.

Accountability Implications of Documentation Gaps

Absence of verifiable documentation creates serious accountability deficits across multiple dimensions

Legal Basis Questioned

The provisional pathway permits registration based on less mature clinical evidence, with ongoing evidence generation and enhanced post-market monitoring during the provisional period. If implementation of the Plan's enhanced monitoring framework cannot be demonstrated from the available record, questions arise regarding the evidentiary basis for assuring that the monitoring commitments associated with the 2021 provisional approvals were delivered, the assurance supporting the 2023 transition decisions, and accountability within the provisional approval pathway.

Public Trust Implications

Australians received COVID-19 vaccines under a provisional framework that explicitly committed to enhanced safety monitoring, and TGA communications emphasised rigorous oversight. Where the record does not document that the Plan's enhanced monitoring was implemented, the public assurances of heightened oversight cannot be independently verified, and the commitments made when reduced pre-market evidence was accepted cannot be shown, on the record, to have been met.

Ministerial Accountability Deficit

The Minister for Health is accountable to Parliament for the TGA's regulatory framework. Where the documentary record does not show what evidence supported transition decisions, how the Plan's enhanced-monitoring commitments were assessed, or whether the promised enhanced monitoring was implemented, the basis on which Parliament can be assured that these commitments were met cannot be independently verified from available records. TGA officials gave Senate evidence (Oct 2025) of 'some difficulty' producing documents by Plan objective, consistent with this documentary gap.

Provisional Approval Framework Integrity

Framework integrity depends on verifiable evidence that enhanced post-market monitoring occurred, that compliance with the Plan's enhanced-monitoring commitments was assessed, and that this assessment was documented before transition. For the COVID-19 vaccine transitions, none of these can be independently verified from the public or FOI record. The audit does not find that oversight did not occur, but that the enhanced monitoring on which the pathway's reduced pre-market requirements rest cannot be verified from available records.

Accountability Implications of Documentation Gaps

Absence of verifiable documentation creates serious accountability deficits across multiple dimensions

Parliamentary Oversight Without Documentation

In Senate estimates no implementation documentation was produced for scrutiny: TGA officials characterised monitoring as 'day-to-day processes' and acknowledged 'some difficulty' producing documents by Plan objective, which sits against the 'enhanced' monitoring commitment; FOI requests were welcomed but no implementation documentation was produced; and no completed implementation review was identified, despite a commitment to review recorded in a February 2022 email. Parliament questioned the TGA, but the documentary basis for assessing whether commitments were met was not available.

International Benchmarks

Australia's regulatory practice is expected to align with international pharmacovigilance good practice (ICH E2E, EMA GVP) and with the transparency standards peer regulators publish. Where documentation meeting these benchmarks is not identified: the alignment between Australia's disclosable record and these benchmarks cannot be demonstrated; the TGA's own COVID-19 monitoring is not documented to the standard it requires of sponsors; and the transparency the pathway relied upon cannot be independently verified.

Commonwealth Law Obligations – Conformity Assessment

Statute / Provision	Requirement	Documentary Finding	Conformity*
PGPA Act 2013, s 37	The accountable authority must cause records to be kept that properly record and explain the entity's performance in achieving its purposes.	No documentation tracking Departmental performance against Plan objectives located (GOV.1)	**Non-conforming**
PGPA Act 2013, s 38 / Rule s 16F(2)	The accountable authority must measure and assess the entity's performance in achieving its purposes; results reported in performance statements.	No performance measurement against Plan outputs located (GOV.2).	**Non-conforming**
Archives Act 1983, s 24	Commonwealth records must not be disposed of except as authorized.	If records existed but were not retained: potential disposal/records management concern. If records were never created: records creation/documentation gap. Available records do not distinguish between these possibilities.	**Indeterminate**
Therapeutic Goods Act 1989, ss 22D, 23AA, 28(2A)(aa)	Provisional determination and registration pathway; conditions may be imposed on provisional registration, including requirements relating to additional evidence generation and post-market monitoring activities	The Plan's enhanced-monitoring framework could not be demonstrated from available documentation to have been operationalised; no documented evaluation against the Plan's enhanced-monitoring commitments was located.	**Conforming †/Documentation gap**
Therapeutic Goods Act 1989, ss 23, 25	Transition to full registration requires an application for registration assessed against statutory safety, efficacy, and quality requirements.	Transition decisions are assessed under s 25 against the general registration requirements; no documented evaluation of implementation of the Plan's enhanced-monitoring commitments was located.	**Conforming †**
FOI Act 1982, s 24A	Take all reasonable steps to locate documents; refuse only where they cannot be found or do not exist.	AICmr 54: TGA took all reasonable steps; documents cannot be found or do not exist.	**Conforming†**
FOI Act 1982, s 24AA(1)(a) vs (1)(b)	Practical refusal must cite the correct ground.	June 2025: request for more than 399 documents identified in scope and refused under s24(1)(b) on practical-refusal grounds (s24AA). OAIC review pending (MR25/01153).	**Under review**

Documentary conformity against Commonwealth obligations; findings reflect the audit's search methodology (sample TRIM searches, FOI, systematic review), rated per ISO 19011:2018.

† Conforming to the general full-registration standard (s 25); that standard does not include assessment against the February 2021 Plan.

**Assessment of apparent non-compliance patterns, subject to formal investigation and legal determination; ratings reflect documentary evidence available to the audit.*

International Pharmacovigilance Standards - Context

IMPORTANT: These are NOT legally binding requirements but provide context for assessing TGA's practices against international norms and relevant international pharmacovigilance benchmarks

Standard	Status/Basis	Key Requirement	TGA Gap
ICH E2E Pharmacovigilance Planning	Recognised good practice; ICH E2E overseas effective date June 2005; adopted by TGA.	Requires structured pharmacovigilance planning, including identification of safety concerns, objectives, actions, rationale, monitoring parameters, and milestones.	No documentation identified demonstrating structured pharmacovigilance action plans aligned with ICH E2E elements, including defined milestones and tracking of planned activities.
CIOMS WG VIII	Internationally recognised good practice.	Traceable audit trails from signal detection to decision. Documented signal investigation protocols and assessment procedures.	No audit trails linking 150 investigated signals to 57 regulatory actions. No complete, dated COVID-specific signal investigation protocol identified (FOI 5082 shows a working draft framework and a work instruction).
EMA GVP Module I & IX	Module I adopted by TGA (1 Aug 2025); Module IX applied as international benchmark.	Quality-system documentation and documented signal management, tracking, assessment and traceability.	No documentation identified meeting EMA GVP Module I & IX quality-system requirements and signal-management for COVID-19 monitoring.
PIC/S Good Vigilance Practices	TGA is PIC/S member (applied here as a benchmark).	Comprehensive pharmacovigilance system documentation, quality management, performance indicators.	Cannot demonstrate PIC/S GVP compliance - no systematic documentation of quality management.
WHO Pharmacovigilance Framework	Recognised international framework.	National pharmacovigilance plans should include objectives, activities, timelines, performance measurement.	No documented performance measurement against plan objectives. No timeline documentation.

Systemic Issues Identified - Pattern Analysis

1. **Documentation vs Activity Gap**

   Activities occurred (150+ weekly reports, 150 safety signals investigated, 57 regulatory actions taken) but systematic documentation of HOW activities were conducted is absent. Creates verification impossibility: outputs visible, processes invisible.

2. **Enhanced vs Standard Monitoring**

   If day-to-day processes were equivalent to enhanced monitoring, the distinct assurance basis of the provisional approval framework is questionable.

3. **Governance Vacuum**

   No governance documentation identified: no implementation oversight, no performance review, no accountability framework. No Feb 2022 review was identified. Four-year accountability gap.* Senate testimony (Oct 2025): A/g Medical Officer acknowledged 'some difficulty' producing documents demonstrating Safety Plan implementation - consistent with records not being systematically organised by objective.

4. **Records Management Failure**

   Systematic absence of implementation documentation raises two possibilities: records were never created, or were created but not retained. Available evidence does not distinguish between them, but either presents a significant records-management concern.

5. **Temporal Contradictions in FOI**

   TGA demonstrated capability (Sept 2024: sample estimate of 2,218+ pages classified by Plan objective.) then claimed impossibility (June 2025: 'too subjective'). Pattern is inconsistent with the earlier demonstrated capability.

6. **Provisional Approval Integrity**

   Cannot verify whether implementation of the TGA's enhanced-monitoring framework was assessed before full approval was granted. Assurance requires verification; verification requires documentation; relevant documentation was not identified. Framework assurance cannot be independently confirmed from available records.

*Cannot demonstrate on public record

Key Findings - Critical Oversight Failures Uncovered

TGA published 150+ weekly safety reports but no documentation identified demonstrates distinguishing enhanced monitoring from routine surveillance. Officials characterised processes as 'day day' indicating no systematic tracking. The absence of documented coordination protocols, completed investigation protocols, and governance mechanisms means the enhanced monitoring underpinning provisional approval cannot be verified from the record, potentially undermining provisional approval framework integrity and public trust.

1. No Evidence of Systematic Enhanced COVID-19 vaccine safety Monitoring

Government promised enhanced surveillance beyond routine processes under the Vaccination Policy. No documented evidence found despite the Department's sample estimate of 2,218+ pages (Sept 2024) and identifying more than 399 documents in scope (FOI 25-0166) refused for processing. Officials described monitoring as 'day-to-day processes' indicating no systematic tracking. No coordination protocols linking AusVaxSafety's 6.8M surveys to TGA's 150 signal investigations located. Enhanced vaccine safety monitoring unverified. (Strategy 2; Senate Oct 2025; QON 559; FOI 25-0166).

2. Complete Absence of Governance and Accountability Frameworks

No oversight, performance measurement or governance mechanisms against the Plan were identified. The Department's sample searches identified no governance documentation in scope. No completed review following the Feb 2022 review commitment, formal evaluation, or audit trail was identified. TGA 2023-24 Annual Report contains zero Plan references despite 68.4 million doses, creating a profound accountability gap. (Outputs GOV.1, GOV.2; TGA Annual Report 2023-24).

3. Signal Detection Systems Only Partly Documented

TGA investigated 150 safety signals and took 57 regulatory actions (Senate Oct 2025). FOI 5082 shows working draft frameworks and methods for signal investigation and cumulative review (outputs 2.2 and 2.3), but no completed protocols, outputs or signal-to-action trail. No documentation links signals to actions or provides traceability against international standards (ICH E2E, CIOMS VIII, EMA GVP IX). No coordination protocols were located between AusVaxSafety surveys and signal investigations (output 2.4).

4. Transparency Failures Undermine Public and Parliamentary Oversight

Despite TGA identifying more than 399 documents in scope (FOI 25-0166) and estimating 2,218+ relevant pages (Sept 2024), TGA consistently refused release citing 'subjectivity' and resource constraints. TGA used identical document classifications systematically for OAIC (Sept 2024) then claimed same classifications 'too subjective' for citizen requests (June 2025). Contradictory statements erode trust and impede parliamentary scrutiny. FOI delays exceeded 1,400 days.

In plain terms: provisional approval accepted reduced pre-market evidence alongside a policy commitment to enhanced post-market safety monitoring. Australians have no way to verify, from the record, that this enhanced monitoring was delivered — or that it differed from routine surveillance at all.

Required Reforms - Restoring Framework Integrity

Public communication (Strategy 4) achieved full implementation; the documentation and governance domains did not. The audit finds systemic gaps requiring substantial reform to make the provisional approval pathway accountable and verifiable in future emergencies.

These same documentation and governance failures will recur in the next pandemic unless addressed. Provisional approval framework integrity depends on learning from COVID-19 experience and implementing reforms for future emergency approvals to ensure public trust.

1. Governance & Accountability

Oversight mechanisms should be strengthened to ensure emergency response frameworks have clear performance measurement and accountability pathways for vaccine safety monitoring. Independent audit and verification processes should underpin regulatory transitions and approvals to demonstrate that enhanced monitoring standards have been satisfied.

2. Documentation & Records Management

Documentation standards should demonstrate how enhanced monitoring differs from standard practice, with protocols, criteria, and key decisions systematically recorded and published. Compliance with mandatory creation, retention, and archiving obligations should be maintained, including during emergencies.

3. Systems for Risk Assessment & Verification

Robust frameworks should be developed for risk-based signal detection, prioritisation, investigation, and causality assessment, with regular scientific reviews and transparent methodologies to support decision-making.

4. Transparency & Public Engagement

Provisional approval compliance should be supported by public access to monitoring data, processes, and methodologies, alongside timely responses to FOI and parliamentary requests. Leadership should be accountable for upholding transparency, documentation and compliance obligations throughout regulatory processes.

5. Emergency Preparedness

Emergency preparedness should include scalable systems for rapid safety monitoring, strong data handling, and requirements for ongoing system improvement for future health emergencies when public trust is most critical.